

PLAB 2 MASTER SERIES

VOLUME TWO

CLINICAL MEDICINE & SPECIALTIES

Clinical Reasoning, Emergency Recognition and Safe Management Across All Specialties

AMaC A. Mansour and Colleagues

AMaC

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United Kingdom

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Dedication

*To the doctors who already know the medicine
but were never taught how to show it.*

*To every candidate who revised the guidelines, memorised the doses,
and still walked out of that room uncertain.*

*This volume is the bridge between what you know
and what examiners need to see.*

HOW TO USE VOLUME TWO

Volume Two builds on the communication, consultation, and core clinical skills developed in Volume One.

This volume focuses on the clinical medicine, specialty, and high-risk presentations commonly encountered in PLAB 2. The emphasis is not on memorising guidelines, but on recognising dangerous conditions, prioritising patient safety, and demonstrating the structured decision-making expected of a UK doctor.

Across ophthalmology, ENT, respiratory medicine, cardiology, diabetes, women's health, paediatrics, psychiatry, surgery, oncology, frailty, safeguarding, and palliative care, the examiner is assessing the same core abilities:

- ✓ Recognise risk early
- ✓ Prioritise ABCDE and patient safety
- ✓ Escalate appropriately
- ✓ Communicate clearly and professionally
- ✓ Reassess after intervention
- ✓ Demonstrate safe NHS practice

Every chapter is structured around examiner expectations and focuses on the actions, verbalisation, and clinical reasoning that generate marks in the examination.

To maximise your score:

1. Read the chapter overview before studying the content.
2. Learn the red flags and escalation triggers.
3. Practise the examiner-facing scripts aloud.
4. Review the Instant Fail sections carefully.
5. Master the Gold Closing Statements.
6. Apply each chapter to mock stations and clinical scenarios.

Remember:

PLAB 2 is not primarily a test of medical knowledge. It is a test of whether you can recognise danger, manage uncertainty, communicate effectively, and keep patients safe.

The candidate who demonstrates safe clinical judgement will consistently outperform the candidate who simply knows the most facts.

AMaC Core Rule:

Recognise Early.

Act Safely.

Escalate Appropriately.

Reassess Frequently.

Think.

Speak.

Perform.

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CHAPTER 27 · ACUTE VISION LOSS & EYE EMERGENCIES

CRAO · GCA · Retinal Detachment · Angle Closure · Amaurosis Fugax

EYE-TIME · CHERRY · HEAD-JAW-VIS · FLASH · PAINFUL

★★★★★ CORE · 🩺 ED/Acute/GP Emergency · ⚠️ HIGH FAIL RISK

Abbreviations used in this chapter: CRAO Central Retinal Artery Occlusion · GCA Giant Cell Arteritis · RAPD Relative Afferent Pupillary Defect · VA Visual Acuity

🧠 Universal OSCE Identity:

"I am assessing acute vision loss using ABCDE, rapid visual triage (EYE-TIME), immediate identification of sight-threatening pathology, and urgent escalation to prevent irreversible blindness or systemic complications."

⚡ Golden Rules:

👉 Visual acuity = GCS of the eye — always first test · 👉 Do NOT delay treatment for investigations (especially GCA/CRAO)

👉 Early escalation is mandatory (ophthalmology ± stroke team)

🚫 Instant Fail Triggers (INSTANT FAIL)

❌ Not checking visual acuity FIRST · ❌ Missing RAPD in CRAO / optic nerve disease
 ❌ Delaying steroids in GCA (blindness risk) · ❌ Treating angle closure as conjunctivitis
 ❌ Dilating a painful/red eye · ❌ Missing retinal detachment (flashes + curtain)
 ❌ Not treating CRAO as stroke equivalent · ❌ No urgent ophthalmology referral · ❌ No SBAR escalation

⚡ Golden 60-Second Flow

0–10s: Safety + call for help + monitoring

10–30s: ABCDE + visual acuity (FIRST test) + glucose

30–60s: IV access + basic bloods + pattern recognition

🔴 Emergency pathology → treat now | 🟢 Stable → investigate after stabilisation

🩺 EYE-TIME Assessment Framework

E	Examine visual acuity FIRST	Use Snellen chart / count fingers / light perception
Y	Why is this emergency?	Time of onset · monocular vs binocular · painful vs painless
E	Examine pupils (RAPD) + fundoscopy	RAPD = afferent defect (optic nerve/retina)
T	Time-critical intervention	Act immediately if CRAO/GCA/angle closure
I	Immediate referral pathway	Ophthalmology ± stroke team

CHAPTER 27 · ACUTE VISION LOSS & EYE EMERGENCIES

M	Medications if indicated	Steroids/pilocarpine/aspirin
E	Escalate + Explain to patient	SBAR + documentation + safety-net



Condition Overview — Quick Recognition

Condition	Key Features	Mnemonic	Immediate Action
CRAO	Sudden painless monocular loss · cherry red spot · RAPD+	CHERRY	Stroke workup + urgent ophthalmology
GCA	Age >50 · temporal headache · jaw claudication · visual symptoms	HEAD-JAW-VIS	Steroids IMMEDIATELY
Retinal Detachment	Flashes + shower of floaters + curtain over vision	FLASH	Same-day ophthalmology — keep still
Acute Angle Closure	Severe eye pain · mid-dilated fixed pupil · foggy cornea · vomiting	PAINFUL	Pilocarpine + acetazolamide — do NOT dilate
Amaurosis Fugax	Transient monocular loss — full recovery	TIA of eye	Aspirin + TIA pathway

● CRAO — Ocular Stroke (CHERRY)

Key Features: Sudden painless monocular vision loss · Cherry red spot on fundoscopy · RAPD positive

Immediate Actions:

- ✓ Urgent ophthalmology referral · ✓ Stroke workup: ECG, carotid Doppler, echocardiogram
- ✓ Consider aspirin if appropriate · ✗ Do NOT wait for spontaneous recovery · ✗ Do NOT treat as isolated eye problem
- 🔧 Examiner Line: “This is an ocular stroke requiring immediate systemic vascular workup.”

● GCA — Large Vessel Vasculitis (HEAD-JAW-VIS)

Key Features: Age >50 · Temporal headache · Jaw claudication · Visual symptoms

Immediate Actions:

- ✓ START steroids immediately — do NOT wait for ESR/biopsy
- ✓ Oral prednisolone 40–60 mg if no visual loss
- ✓ IV methylprednisolone 500 mg–1 g + same-day ophthalmology if visual symptoms
- ✓ Temporal artery biopsy within 1 week
- ✗ Do NOT delay steroids for ESR · ✗ Do NOT wait for biopsy results
- 🔑 **Examiner Line:** “GCA until proven otherwise — steroids must be started immediately to prevent irreversible blindness.”

👁️ Retinal Detachment — FLASH

Key Features: Flashes of light · Shower of floaters · Curtain/shadow over vision

Immediate Actions:

- ✓ SAME-DAY ophthalmology referral · ✓ Advise patient to keep still
- ✗ Do NOT dilate pupil · ✗ Do NOT delay referral
- 🔑 **Examiner Line:** “This is retinal detachment until proven otherwise — time-sensitive surgical emergency.”

👁️ Acute Angle Closure — PAINFUL

Key Features: Severe eye pain · Mid-dilated fixed pupil · Foggy cornea · Nausea/vomiting

Immediate Actions:

- ✓ Pilocarpine drops · ✓ Acetazolamide · ✓ Analgesia + antiemetics · ✓ Urgent ophthalmology referral
- ✗ Do NOT dilate eye · ✗ Do NOT treat as conjunctivitis
- 🔑 **Examiner Line:** “Acute angle closure glaucoma requiring immediate pressure reduction and urgent ophthalmology intervention.”

👁️ Amaurosis Fugax — TIA of the Eye

Key Features: Brief transient monocular vision loss · Full recovery

Immediate Actions:

- ✓ Stroke workup: ECG, carotid Doppler, echocardiogram · ✓ Aspirin · ✓ Same-day TIA pathway
- ✗ Do NOT dismiss as benign
- 🔑 **Examiner Line:** “This is a TIA equivalent and carries a high early stroke risk.”

📡 Escalation — SBAR

“I am escalating early to ophthalmology and relevant specialty teams due to time-critical risk of permanent vision loss or systemic vascular pathology.”

S	Situation	Acute vision loss with suspected [condition]
B	Background	Risk factors: age, AF, HTN, diabetes, myopia
A	Assessment	VA reduced, RAPD ±, funduscopy findings
R	Recommendation	Urgent ophthalmology ± stroke team review



Examiner Checklist

- ✓ Visual acuity first · ✓ RAPD assessed · ✓ Fundoscopy described
- ✓ CRAO identified + stroke workup · ✓ GCA treated immediately with steroids
- ✓ Retinal detachment referred same day · ✓ Angle closure treated (no dilation)
- ✓ SBAR escalation done · ✓ Safety-net + explanation given



8-Minute OSCE Timeline

0–1 min	Recognition + VA first
1–2 min	ABCDE + pupil exam
2–3 min	Fundoscopy + pattern recognition
3–4 min	Immediate treatment
4–5 min	Referral decision
5–6 min	Investigations
6–7 min	Escalation + explanation
7–8 min	Reassessment + summary



Final Exam Mantra

VA → RAPD → FUNDOSCOPY → CHERRY / FLASH / PAINFUL → TREAT → REFER → ESCALATE

★ Gold Closing Statement:

"I performed ABCDE and prioritised visual acuity as the first test. I identified a sight-threatening emergency using EYE-TIME. Immediate management: steroids for GCA, stroke workup for CRAO, pressure-lowering treatment for angle closure, and urgent referral for retinal detachment. Escalated to ophthalmology and relevant teams with continuous reassessment to prevent permanent vision loss."

VA first always GCS of the eye	RAPD positive CRAO / optic nerve	Cherry red spot CRAO — ocular stroke	GCA steroids now don't wait for ESR	IV steroids + ophth GCA with visual loss
FLASH = detachment same-day referral	No dilation angle closure rule	Pilocarpine + acetaz angle closure treatment	Amaurosis fugax TIA of eye — stroke risk	SBAR escalate named team always

TOP 5 EXAMINER Q&A · ACUTE VISION LOSS & EYE EMERGENCIES

Q1. Sudden painless vision loss in one eye. First diagnosis to exclude?	Central retinal artery occlusion or GCA-related ischaemia; both are time-critical. Same-day ophthalmology and GCA screen.
Q2. Which features point to giant cell arteritis?	Age over 50, temporal headache, scalp tenderness, jaw claudication, raised ESR/CRP; start high-dose steroids immediately.

TOP 5 EXAMINER Q&A · ACUTE VISION LOSS & EYE EMERGENCIES

Q3. Painful red eye with haloes and a fixed mid-dilated pupil?	Acute angle-closure glaucoma: lie flat, analgesia and antiemetic, urgent same-day ophthalmology for pressure-lowering treatment.
Q4. Flashes, floaters and a curtain over the vision?	Retinal detachment until proven otherwise: urgent same-day ophthalmology referral; do not reassure and discharge.
Q5. What must every eye station include?	Visual acuity in each eye separately: it is the vital sign of the eye and a marked step.
CURVEBALL	The patient with sudden vision loss mentions jaw pain on chewing. Treat as GCA: steroids now, bloods after; sight in the other eye is at risk.

CHAPTER 28 · UPPER AIRWAY & STRIDOR

Epiglottitis · Croup · Foreign Body · Angioedema
AIRWAY FIRST · SAD C · UP HEALS · CICV

★★★★★ CRITICAL · 🚨 RED PATH · ⚠️ AIRWAY INSTANT FAIL DOMAIN

Abbreviations used in this chapter: CICV Cannot Intubate, Cannot Ventilate

🧠 Universal OSCE Identity:

"I am managing an upper airway obstruction using immediate airway-safe positioning, early escalation to anaesthetics, avoidance of distressing interventions, and structured differential diagnosis using a RED PATH safety-first approach."

⚡ Golden Rules:

- ▶ Airway obstruction is treated before diagnosis · ▶ Distress can precipitate complete airway collapse
- ▶ Epiglottitis = DO NOT DISTURB · ▶ Anaesthetics call is immediate, not conditional

⊘ Instant Fail (INSTANT FAIL)

- 🔥 No recognition of stridor as airway emergency · 🔥 Throat examination in suspected epiglottitis
- 🔥 Lying patient flat · 🔥 Delayed anaesthetics call · 🔥 Forced oxygen mask causing distress
- 🔥 Blind finger sweep in foreign body · 🔥 Separation from parent (paediatrics)
- 🔥 Sedation in airway compromise · 🔥 Missing quiet stridor (late deterioration sign)
- 🔥 Attempting cannulation before airway control

⚡ Golden 60-Second Opening Flow

0–10s: Doorway assessment — do NOT approach

10–30s: Identify stridor + keep upright + parent present

30–60s: Call anaesthetics + prepare airway trolley

🔊 Must-Say: "This is stridor indicating upper airway obstruction. I will assess from the doorway, keep the patient upright, avoid distress, and call anaesthetics immediately."

🧠 AIRWAY FIRST Framework

A	Assess from distance	Doorway only — do NOT approach
I	Immediate anaesthetics call	Not conditional — call NOW
R	Recognise RED PATH	Severity assessment
W	Work of breathing	Stridor severity + accessory muscles
A	Avoid throat exam / avoid supine	Epiglottitis — never examine throat
Y	Your role = buy time	UP HEALS while awaiting expert help
F	Family present	Calm = airway protection — keep parent

I	Involve ENT early	Second call after anaesthetics
R	Request difficult airway trolley	Ready before needed
S	SAD C differential	Epiglottitis · Angioedema · Diphtheria · Croup · Bacterial tracheitis (Staph aureus — severe stridor, toxic, age <6y, needs ITU/anaesthetics)
T	Theatre preparation	Prepare for definitive airway

 **Must-Say:** “I am using AIRWAY FIRST to ensure safe escalation and avoidance of airway-triggering interventions.”

abc ABCDE — Airway-Dominant Structure

	Component	Assess	Script / Action
A	Airway	Drizzling · voice change · stridor	"I will not approach or examine the throat if epiglottitis is suspected."
B	Breathing	RR · SpO ₂ · accessory muscles	"Quiet stridor may indicate deterioration rather than improvement."
C	Circulation	Pulse only	Avoid distressing BP/cannulation
D	Disability	Agitation = airway risk	Do NOT sedate — agitation is protective
E	Exposure	Brief only · vaccination status	Minimal intervention

SAD C — Differential Diagnosis

Condition	Key Features	Immediate Action
Epiglottitis	Drizzling · tripod position · toxic appearance · no cough	DO NOT DISTURB + anaesthetics NOW
Angioedema	Tongue/lip swelling · ACE inhibitor · rapid onset	IM adrenaline + airway escalation
Diphtheria	Grey membrane · unvaccinated · bull neck	Senior + airway protection · antitoxin
Croup	Barking cough · viral · usually non-toxic child	Dexamethasone ± nebulised adrenaline
Foreign body	Sudden choking · witnessed episode	Obstruction algorithm — NEVER blind finger sweep

Condition Management — Details

● Epiglottitis — Highest Risk:

- ✓ Upright position only · ❌ Do NOT examine throat · ❌ Do NOT cannulate if distressing
- ✓ Keep parent present · ✓ Immediate anaesthetics call · ✓ Prepare theatre intubation
- ⚠ Quiet stridor = impending collapse (NOT improvement)

● Croup:

- Barking cough · usually non-toxic · Management:
- ✓ Oral dexamethasone · ✓ Nebulised adrenaline if moderate/severe · ✓ Keep calm + upright
- ⚠ Quiet stridor = exhaustion (RED FLAG)

● Foreign Body:

Sudden onset choking episode. Algorithm:

- Partial obstruction → encourage cough · Complete obstruction → back blows / abdominal thrusts · Failure → CPR pathway
- ❌ NEVER blind finger sweep.

● Angioedema:

- Tongue/lip swelling · ACE inhibitor often present · Management:
- ✓ IM adrenaline if airway risk · ✓ No routine antihistamine/steroid (RCUK 2021) · ✓ Stop ACE inhibitor · ✓ Early anaesthetics call

🕒 Buying Time — UP HEALS

U	Upright position	Optimise airway geometry
P	Parent present	Calm child = reduced airway spasm
H	Heliox (if available)	Reduces airway resistance
E	Nebulised adrenaline	Reduces mucosal oedema
A	Avoid distress	Distress → airway spasm → collapse
L	Low stimulation environment	Quiet, calm, dim
S	Steroids (croup/angioedema)	Dexamethasone or hydrocortisone

🎤 **Must-Say:** “I am using UP HEALS to stabilise while awaiting definitive airway management.”

📶 Escalation Pathway — Fixed Order

①	Anaesthetics	Immediate — not conditional
②	ENT	Second call
③	ICU	If deteriorating
④	Theatre	Prepare for definitive airway

Prepare: Difficult airway trolley · Intubation equipment · Surgical airway readiness

CICV — Last Resort Airway

Indication: Cannot intubate + Cannot ventilate

Procedure: Scalpel–bougie–tube technique (surgical cricothyroidotomy)

 **Must-Say:** “I am prepared for cricothyroidotomy in a CICV scenario as a last-resort life-saving intervention.”

Communication — Examiner Marks Core

“Please stay with your child and keep them upright. Your calm presence is helping maintain their airway. We are treating this as an airway emergency and senior help is arriving immediately.”

Deterioration Signs — Red Flags

Quiet stridor · Reduced agitation (fatigue) · Falling SpO₂ · Cyanosis · Silent chest
Reduced work of breathing = EXHAUSTION, NOT IMPROVEMENT

 “Less noise does not mean improvement — it may indicate airway failure.”

Examiner Checklist

- ✓ Stridor recognised as airway emergency · ✓ Doorway assessment performed · ✓ AIRWAY FIRST verbalised
- ✓ Immediate anaesthetics call made · ✓ No throat exam in epiglottitis · ✓ Upright positioning
- ✓ Parent kept with child · ✓ SAD C used correctly · ✓ UP HEALS applied
- ✓ Correct foreign body algorithm · ✓ Correct angioedema treatment · ✓ CICV awareness stated
- ✓ Deterioration signs identified

Final Exam Mantra

Stridor = RED PATH → Doorway → Anaesthetics → No throat exam → Upright → SAD C → UP HEALS → Prepare airway failure

★ **Gold Closing Statement:**

“This is upper airway obstruction until proven otherwise. I assessed from the doorway, maintained airway-safe positioning, avoided distressing interventions, escalated immediately to anaesthetics, applied the SAD C differential, initiated UP HEALS for stabilisation, and prepared for definitive airway management including ENT involvement and potential surgical airway if required.”

Stridor = red path immediate response	Doorway assess never approach first	Anaesthetics NOW not conditional	No throat exam epiglottitis rule	Upright always never lay flat
Parent with child calm = airway	Quiet stridor worse not better	SAD C differential structured approach	UP HEALS buy time safely	CICV ready last resort airway

TOP 5 EXAMINER Q&A · UPPER AIRWAY & STRIDOR	
Q1. What does stridor at rest tell you?	Significant upper airway obstruction: do not distress the patient, give oxygen, and call anaesthetics and ENT immediately.
Q2. Suspected epiglottitis. What must you avoid?	Examining the throat or lying the patient flat; keep them calm and upright with senior airway help on the way.
Q3. How do you treat moderate croup?	Single dose of oral dexamethasone; nebulised adrenaline with senior input if severe.
Q4. Complete choking obstruction in an adult?	5 back blows, then 5 abdominal thrusts, alternating; CPR if they become unconscious.
Q5. Anaphylaxis vs other causes of stridor?	Look for rapid onset, urticaria, lip and tongue swelling, and a trigger: adrenaline 0.5 mg IM without delay.
CURVEBALL	A child with suspected epiglottitis cries when you approach with a tongue depressor. Stop: examination can precipitate complete obstruction; await senior airway support.

CHAPTER 29 · ACUTE UROLOGICAL & SCROTAL EMERGENCIES

Torsion · Retention · Fournier's · Urosepsis · Paraphimosis

★★★★★ CORE · 🟡 Surgical Acute Take · ⚠️ CRITICAL FAIL RISK

Abbreviations used in this chapter: AKI Acute Kidney Injury · LMWH Low Molecular Weight Heparin · VTE Venous Thromboembolism

🧠 Universal OSCE Identity:

"I manage time-critical urological emergencies using ABCDE, rapid recognition of life-threatening pathology, immediate escalation, and safe stabilisation – avoiding harmful delays."

Examiner Focus: Speed of recognition · Early escalation (before full diagnosis) · Avoidance of dangerous actions · Safety-first decisions

⚡ Master Spine — IDENTIFY → ACT → ESCALATE

IDENTIFY → RED / STABLE → ACT (1–2 min) → ESCALATE → REASSESS

Path	Condition	Action
🔴 RED	Torsion · Fournier's · Urosepsis+obstruction · Paraphimosis (failed reduction) · Retention+sepsis/AKI	Act NOW + Escalate
🟢 STABLE	Epididymo-orchitis · mild infection · painless chronic retention	Investigate then treat

🚫 Instant Fail Triggers (NEVER / ALWAYS)

❌ NEVER	✅ ALWAYS
Wait for ultrasound in torsion	Call urology within 2 minutes
Catheterise with blood at meatus / pelvic fracture	Stop – NO catheter
Miss absent cremasteric reflex	Check both sides
Delay antibiotics in sepsis	Sepsis Six within 1 hour
Miss Fournier's extension	Examine perineum + perianal + buttocks + inner thighs
Discharge suspected torsion	Escalate + admit

⚡ Golden 60-Second Flow

Time	Action
0–10s	ABCDE + identify emergency (sudden pain? vomiting?)
10–20s	RED flags: absent cremasteric? high-riding testis?

Time	Action
20–30s	TWIST score (≥ 5 = theatre; 3–4 = urology + US only if no delay)
30–45s	CALL urology (torsion/Fournier's) OR SEPSIS SIX (urosepsis)
45–60s	Manual detorsion / paraphimosis reduction / safe catheter

● Testicular Torsion — Time is Testis

TWIST score ≥ 5 → theatre immediately — no imaging delay.

TWIST 3–4 → urology + ultrasound only if no delay to surgical review.

Manual Detorsion:

Left testis → rotate clockwise (from patient's view) · Right testis → rotate counter-clockwise

180–360 degrees. If pain increases → reverse direction.

Consent: "Bilateral orchidopexy – contralateral testis fixed (bell-clapper is bilateral in 40–50%)."

⚠ TRAP: Previous orchidopexy does NOT exclude torsion.

● Fournier's Gangrene — Surgical Emergency

FOUR D's: Discolouration · Deep pain (out of proportion) · Debris/crepitus · Deterioration

Examine: perineum + perianal + buttocks + inner thighs — all areas.

Action:

✓ Immediate surgical referral (urology + general surgery) · ✓ IV antibiotics + fluids

✓ LRINEC score (≥ 6 high risk — but normal does NOT exclude) · ✓ Check blood glucose (40–60% diabetic)

● Urosepsis — Sepsis Six + Source Control

Sepsis Six (within 1 hour):

Oxygen → Blood cultures → IV antibiotics → IV fluids → Lactate → Urine output

Source Control = RELIEVE OBSTRUCTION: Catheter / stent / nephrostomy OR drain abscess

Add: VTE assessment + prophylactic LMWH (sepsis = pro-thrombotic)

● Paraphimosis — Reduce Before Calling

Analgesia first: Dorsal nerve block (2% lignocaine no adrenaline)

Step	Technique	Notes
1	Ice	Reduces oedema
2	Compression	Wrap/squeeze glans to reduce swelling
3	Osmotic reduction	Sugar or glucose gauze over glans
4	Manual reduction	Push glans through foreskin
5	Dundee technique	Multiple needle punctures to release oedema

Step	Technique	Notes
6	Failed → urology	Dorsal slit or circumcision

● Acute Urinary Retention — RED if Sepsis/AKI

Safe Catheterisation — 5 Cs:

Check urethral injury · Correct size · Clean aseptic · Cuff with sterile water ONLY · Collect urine

High PVR >800 mL: Consider catheter valve or intermittent catheterisation

Post-obstructive diuresis: Monitor BP/electrolytes — do NOT clamp (not evidence-based)

✗ NEVER catheterise with blood at meatus or suspected pelvic fracture.

● Green Path — Stable Conditions

Condition	Action
Epididymo-orchitis	Urinalysis + MSU + STI screen (<35) + antibiotics (cover STI or UTI)
Painless chronic retention	Elective catheterisation or trial without catheter
Mild infection	Outpatient care

📢 Escalation Line — Say Exactly

"I am escalating immediately to [urology/general surgery] due to a time-critical emergency. This is [torsion/Fournier's/sepsis/paraphimosis]. I have [immediate action]. I will reassess."

📋 Examiner Checklist

- ✓ Time documented · ✓ Torsion recognised early + TWIST score
- ✓ Urology called immediately — no imaging delay · ✓ Manual detorsion attempted
- ✓ Urethral injury excluded — safe catheterisation · ✓ Sepsis Six + source control
- ✓ Fournier's full perineal examination · ✓ Paraphimosis reduced or escalated
- ✓ Escalation verbalised · ✓ Reassessment stated

🎯 Final Exam Mantra

Torsion → call before palpating.

Fournier's → examine before referring.

Urosepsis → antibiotics before cultures.

Paraphimosis → reduce before calling.

Retention → check urethra first.

★ Gold Closing Statement:

"I identified a time-critical urological emergency. I escalated immediately – torsion called urology within 2 minutes without imaging. Fournier's – full perineal exam + surgical referral. Urosepsis – Sepsis Six with source control. Paraphimosis – reduction with analgesia. I will reassess after every intervention."

CHAPTER 29 · ACUTE UROLOGICAL & SCROTAL EMERGENCIES

Torsion call urology before imaging	TWIST ≥5 theatre immediately	Bell-clapper bilateral orchidopexy	Fournier's examine perineum fully	FOUR D's Discolouration · Deep pain · Debris · Deterioration
Urosepsis Sepsis Six + source control	No catheter blood at meatus rule	Paraphimosis reduce before calling	5 Cs catheter safe catheterisation	Post-obstructive monitor, don't clamp

TOP 5 EXAMINER Q&A · ACUTE UROLOGICAL & SCROTAL EMERGENCIES	
Q1. Acute scrotal pain in a teenager. Default diagnosis?	Testicular torsion until proven otherwise: urgent urology referral; the window is roughly 6 hours.
Q2. Does a normal cremasteric reflex exclude torsion?	No clinical sign reliably excludes it; if suspected, refer for surgical exploration rather than waiting for imaging.
Q3. How do you manage acute urinary retention?	Catheterise, record the residual volume, treat the cause, and check renal function.
Q4. What is Fournier's gangrene and the key action?	Necrotising infection of the perineum: IV antibiotics and immediate surgical debridement; senior escalation at once.
Q5. Infected, obstructed kidney. Why is it an emergency?	Pus under pressure destroys the kidney and drives sepsis: urgent decompression by nephrostomy or stent.
CURVEBALL	Ultrasound is available in two hours for suspected torsion. Do not wait: this is a clinical diagnosis and delayed exploration loses the testis.

CHAPTER 30 · CARDIOVASCULAR RISK & DIABETES ANNUAL REVIEW

6-PACK · TARGET · LOOK-FEEL · M-S-G-I · DRIVE-5 · SADMAN

HbA1c Individualisation · QRISK3 · Foot Examination · SGLT2i/GLP-1 · Insulin Safety · Shared Decision-Making

★★★★★ **CRITICAL** · **CORE STATION** · **△ HIGH FAIL RISK**

Abbreviations used in this chapter: **AKI** Acute Kidney Injury · **DKA** Diabetic Ketoacidosis · **DVLA** Driver & Vehicle Licensing Agency · **GLP-1** Glucagon-Like Peptide-1 agonist · **QRISK3** QRISK3 cardiovascular risk score · **SGLT2** Sodium-Glucose Cotransporter-2 inhibitor

🎯 Core Exam Focus:

Complication screening · Cardiovascular risk management · Medication safety and escalation · Lifestyle and social tailoring · Shared decision-making

👁️ **Examiner Lens:** "Can this candidate deliver safe, structured, patient-centred long-term diabetes care?"

Marks come from: Verbal structure · Prioritisation · Safety-netting · Communication clarity · Clinical completeness

💡 Why Candidates Fail:

👁️ Same HbA1c target for all patients · 🚫 No foot examination · 🚫 Missing Charcot foot

🚫 No insulin driving advice · 🚫 Unsafe sick day advice · 🚫 Poor cardiovascular risk explanation

🚫 No social tailoring · 🚫 No shared decision-making · 🚫 No safety netting

🧠 **Core Rule:** Diabetes annual review = complications + cardiovascular risk + safety + individualisation.

🔴 Golden Opening Script

"Welcome to your diabetes annual review. Today I'd like to review your blood sugar control, cardiovascular risk, medications, and check for complications affecting your eyes, kidneys, and feet. We'll also discuss safety issues like hypos, driving, and sick day rules, and agree a plan together."

"Are there any particular concerns you'd like us to focus on today?"

"Before we begin, could you talk me through your current diabetes medications and how you are taking them?"

📋 6-PACK Framework

P	Prioritise Targets (HbA1c)	Individualise using TARGET
A	Assess Complications	Eyes · Kidneys · Feet
C	Cardiovascular Risk	QRISK3 · BP · Cholesterol · Smoking
K	Knowledge & Safety	Hypoglycaemia · Sick day rules · Driving
E	Escalate Treatment	M-S-G-I pathway

T	Tailor Care	Occupation • Cost • Literacy • Culture • Fasting • Disability
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HbA1c Individualisation — TARGET

T	Treatment burden	Complexity of current regimen
A	Age	Older patients → more relaxed targets
R	Risk of hypoglycaemia	Driver, lives alone, impaired awareness
G	General frailty	Frail → relaxed target
E	Existing complications	Established CVD → individualise
T	Time to benefit	Limited life expectancy → relaxed target

Exam Anchor: “For many adults, NICE often uses HbA1c targets around 48 mmol/mol, but targets must always be individualised using TARGET.”

 **Script:** “Your HbA1c is [X], and the trend is [improving/stable/worsening]. Using TARGET, I would individualise your goal based on your overall health and risk of hypoglycaemia. What feels realistic for you?”

Cardiovascular Risk — QRISK3

“Your QRISK3 score estimates your risk of heart attack or stroke over the next 10 years.”
100-Person Explanation: “A risk of [X]% means that out of 100 people similar to you, about [X] may have a heart attack or stroke in the next 10 years.”

Statin Discussion: “Statins significantly reduce cardiovascular risk. The decision is shared and based on what matters to you.”

Blood Pressure: “Your BP is [X]. Controlling blood pressure protects your heart, brain, kidneys, and eyes.”

Foot Examination — LOOK-FEEL

L	LOOK: Inspect	Toes • Soles • Heels • Pressure areas
O	Observe footwear	“Let’s check your footwear — poorly fitting shoes commonly cause ulcers.”
O	One-minute monofilament	Loss of sensation = neuropathy risk
K	Key pulses	Dorsalis pedis • Posterior tibial
F	Feel temperature	Warm = infection/Charcot • Cold = vascular disease
E	Evaluate risk	Low • Increased • High
E	Educate	Self-care + footwear + warning signs

**Charcot Foot — Emergency:**

Red · 🔥 Hot · 💧 Swollen · Painless

"This is suspicious for Charcot foot, which is an emergency. Do not weight-bear. I am arranging urgent specialist foot team review today."

**Medication Escalation — M-S-G-I**

	Drug	Key Script
M	Metformin	First-line — check eGFR/tolerability
S	SGLT2 inhibitor	"This medication protects the heart and kidneys beyond glucose control."
G	GLP-1 receptor agonist	"Helps with blood sugar and weight. Nausea common initially. Some are once-weekly injections."
I	Insulin	Check driving before starting. NEVER stop during illness — dose adjust instead.

**Driving Rules — DRIVE-5****Ask first:** "Do you drive?"

1	Test BG ≥ 5.0 mmol/L before EVERY journey. Do NOT drive if BG < 5.0 mmol/L. Check every 2 hours on long journeys. Always carry rapid-acting glucose in the vehicle. NOTIFY DVLA if insulin-treated (mandatory).	Every time
2	Recheck every 2 hours	Long journeys
3	Hypo while driving → STOP immediately	Pull over safely
4	Hypo while driving: STOP safely, remove keys, move to passenger seat. Treat hypo. Wait 45 MINUTES minimum before driving again (not just until BG normalises). Document episode.	Not 15 — 45 minutes
5	Carry glucose in vehicle at all times	Mandatory

Hypoglycaemia Treatment — FAST 15-20:

15–20g fast-acting carbohydrate (glucose tablets, juice) · Recheck BM in 15 minutes

**Sick Day Rules — SADMAN**

	Drug	Action During Illness
S	SGLT2 inhibitor	STOP — risk of DKA during illness (including euglycaemic DKA — glucose may be only mildly elevated; do not rely on glucose alone to exclude DKA when on SGLT2 inhibitor)
A	ACEi/ARBs	STOP — AKI risk
D	Diuretics	STOP — dehydration risk
M	Metformin	STOP — lactic acidosis risk

	Drug	Action During Illness
A	Monitor	Check glucose more often during illness
N	NSAIDs	STOP — AKI risk

**Insulin Safety Addition:**

“If you take insulin, do NOT stop it completely during illness. You may need dose adjustment, but stopping insulin entirely can cause dangerous hyperglycaemia or diabetic ketoacidosis.”

**Social & Cultural Care**

Ask about: Smoking · Occupation · Cost · Interpreter · Literacy · Fasting (Ramadan)

“Smoking significantly increases cardiovascular risk. Would you like support to stop?”

“Healthy eating does not need to be expensive — we can adapt advice to your situation.”

“Are you planning to fast for Ramadan? We can adjust your medication safely.”

**Complications Screening · Vaccinations**

Eyes: “When was your last diabetic retinal screening? Usually annual. If overdue, I will refer you.”

Kidneys: “I’d review eGFR and urine albumin-to-creatinine ratio (ACR) to detect early kidney disease and guide medication choice.”

Vaccinations: “Annual flu vaccination and appropriate pneumococcal vaccination recommended.”

Teach-back: “Just to check I’ve explained clearly — could you tell me what you would do if you had a hypo?”

**Follow-Up & Safety Netting**

Situation	Follow-Up Interval
Stable, well-controlled	Annual review
Medication change	3 months
Poor control	3–6 months
High-risk feet	More frequent podiatry
New insulin	Early review

Safety Netting — Return if:

Recurrent hypos · Foot redness/swelling · Vomiting or dehydration · DKA symptoms · Unexpected weight loss

**Instant Fail Behaviours**

No HbA1c individualisation ·
 No foot exam ·
 Missed Charcot foot
 Unsafe insulin advice ·
 No driving assessment ·
 Poor QRISK explanation
 No social tailoring ·
 No safety netting

 Final Exam Mantra

6-PACK → TARGET HbA1c → QRISK3 → LOOK-FEEL feet → M-S-G-I escalation → DRIVE-5 → SADMAN → Social tailoring → Teach-back → Safety-net

★ **Gold Closing Statement:**

"To summarise, your HbA1c is [X] and we have individualised your target using TARGET. Your cardiovascular risk has been discussed using QRISK3, and we have reviewed your medications, kidney function, and foot health. We have also covered safety issues including hypoglycaemia, driving rules, and sick day guidance. Our plan is [X], with follow-up in [Y]. Please seek help earlier if any red flag symptoms develop. Do you have any questions?"

6-PACK framework structured annual review	TARGET HbA1c never one-size-fits-all	LOOK-FEEL feet every annual review	Charcot = emergency no weight-bearing	M-S-G-I escalation stepwise treatment
DRIVE-5 insulin + driving rules	SADMAN sick day stop these 4 drugs	Insulin never stop dose adjust, not stop	QRISK3 + statins shared decision	Safety-net always red flags + follow-up

TOP 5 EXAMINER Q&A · CARDIOVASCULAR RISK & DIABETES ANNUAL REVIEW

Q1. What does QRISK3 estimate and what follows a score of 10% or more?	Ten-year cardiovascular risk; discuss and offer atorvastatin 20 mg alongside lifestyle change, as a shared decision.
Q2. How do you individualise an HbA1c target?	Balance age, frailty, hypoglycaemia risk and comorbidity: roughly 48 for the young and fit, relaxed to 53 or higher in frailty.
Q3. What does the diabetic foot check include?	Inspection, foot pulses and 10 g monofilament sensation, with risk stratification and podiatry referral as indicated.
Q4. Which annual screens does a diabetic review include?	Retinal screening, urine ACR and renal function, feet, blood pressure, lipids and HbA1c.
Q5. The patient declines a statin. What do you do?	Explore concerns, give absolute-risk information, respect the decision, document it, and keep the door open for review.
CURVEBALL	The patient asks "will I be on these tablets forever?" Explain honestly: treatment is long-term while risk persists, but always reviewable: this is a partnership, not a sentence.

CHAPTER 31 · RESPIRATORY REVIEWS

Asthma & COPD Annual Management

SALT · PRESSED · SMILE · BREATHE · MART · GOLD · Three As · Ask–Advise–Act

★★★★★ CORE · 🧑 GP / Respiratory Clinic · ⚠️ HIGH FAIL-RISK

Abbreviations used in this chapter: COPD Chronic Obstructive Pulmonary Disease · DPI Dry Powder Inhaler · DVLA Driver & Vehicle Licensing Agency · LABA Long-Acting Beta-Agonist · MART Maintenance & Reliever Therapy · MRC Medical Research Council scale · SABA Short-Acting Beta-Agonist

🎯 **Core Exam Idea:** Respiratory review = Control + Technique + Adherence + Safety + Stepwise escalation

👁️ **Examiner Lens:** “Can this candidate optimise inhalers safely, prevent exacerbations, and recognise deterioration early?”

Marks come from: Inhaler technique · Recognising poor control · Safe escalation · Smoking intervention · Patient education · Action planning · Safety-netting

🧠 **Core Rule:** Most uncontrolled asthma/COPD is caused by poor technique, poor adherence, or smoking — not lack of medication.

💣 Why Candidates Fail:

💣 No inhaler technique check · 💣 Assume adherence is good · 💣 Step up without Three As

💣 Forget rinse & spit after ICS · 💣 Miss pMDI vs DPI distinction · 💣 No smoking discussion

💣 No action plan · 💣 Unsafe MART explanation · 💣 No safety-netting

🔴 Golden Opening Script

“Thank you for coming in today. I'd like to review your breathing symptoms, check how well your inhalers are working and being used, and see whether any treatment changes are needed to help prevent flare-ups and keep you well.”

“I'll also ask about smoking, triggers, and safety issues. Is that okay?”

🕒 8-Minute Review Flow

0–1 min	Introduction + purpose
1–2 min	Assess symptom control
2–3 min	Inhaler technique check (MAJOR MARK AREA)
3–4 min	Adherence + smoking + triggers
4–5 min	Three As before escalation
5–6 min	Stepwise management
6–7 min	Education + action plan
7–8 min	Safety-net + follow-up

1 Assess Control

Ask About	Red Flags
Daytime symptoms	Increasing reliever use
Night waking	Night symptoms
Reliever use frequency	Recent hospital admission
Exercise limitation	Frequent oral steroids
Exacerbations + admissions	Worsening exercise tolerance

Controlled Asthma Anchor: Reliever ≤ 2 /week · No night symptoms · Minimal activity limitation

 **Must-Say:** “Frequent flare-ups or increased reliever use suggest poor control and mean we should review treatment and inhaler use carefully.”

2 Inhaler Technique — PRESSED (pMDI) vs DPI

 Golden Rule: Never assume inhaler technique is correct.

 Always say: “Please show me how you use your inhaler.”

	PRESSED (pMDI)	DPI	Common Errors
P	Prepare: shake + remove cap	Load dose (twist/click)	Not shaking pMDI
R	Respire out gently	Breathe out gently	Breathing out into mouthpiece
E	Engage: seal lips	Seal lips	Poor lip seal
S	Squeeze: press at start of inhale	N/A — no actuation	Pressing before inhaling
S	Slow deep breath in	QUICK deep breath in	pMDI too fast · DPI too slow
E	Expire: hold breath 5–10s	Hold breath 5–10s	Not holding breath
D	Do again: wait 30–60s	Wait before repeat	Multiple puffs together

 ICS Safety — Automatic Mark Phrase: “After using your steroid inhaler, rinse your mouth and spit out the water to prevent oral thrush.”

3 Spacer Education — SMILE

When to Use: pMDI users · Poor coordination · Recurrent exacerbations · High-dose ICS

S	Shake + insert	Shake inhaler and insert into spacer
M	Make seal	Seal lips around spacer mouthpiece

I	Inhale slowly	One puff → breathe in slowly
L	Listen	Whistle = breathing in too fast
E	Exhale away	Breathe out away from mouthpiece

Cleaning: Wash weekly in warm soapy water · Air dry naturally ·  Do NOT towel dry

④ Three As — Before Any Step-Up

 **Must-Say Safety Rule:** Before increasing treatment, always check Three As first.

A	Adherence	"Are you taking it regularly?"
A	Assessment of technique	"Can you show me how you use it?"
A	Alternative diagnosis	"Could another diagnosis or trigger be contributing?"

⑤ Asthma Management — SALT

	SALT	Detail
S	SABA only	Rare — only if symptoms are infrequent
A	ADD ICS	Low-dose inhaled corticosteroid
L	ADD LABA	Combination ICS/LABA inhaler
T	THINK MART	One inhaler as BOTH preventer and reliever

MART Safety Scripts:

"Your combination inhaler is now your main preventer and reliever inhaler. You usually no longer need routine blue inhaler use."

"Keep your blue inhaler as a backup in case your MART inhaler is unavailable."

Step-Down Rule: Only consider stepping down if stable ≥3 months. Reduce gradually.  Do NOT stop ICS abruptly.

⑥ Action Plan — Green / Amber / Red

Zone	Status	Action
 GREEN	Well controlled	Continue usual treatment
 AMBER	Worsening symptoms	Increase treatment as advised
 RED	Severe symptoms / poor response	Seek urgent medical help

 **Must-Say Safety-Net:** "Please seek urgent help if your inhaler is not relieving symptoms, you are struggling to speak, or your breathing rapidly worsens."

⑦ COPD Review — BREATHE + GOLD Staging

Confirm Diagnosis: "COPD is confirmed by post-bronchodilator spirometry showing FEV1/FVC below 0.7."

GOLD Stage	FEV1 %	Severity
GOLD 1	≥80%	Mild
GOLD 2	50–79%	Moderate
GOLD 3	30–49%	Severe
GOLD 4	<30%	Very severe

	BREATHE	Detail
B	Bronchodilators	SABA/SAMA → LABA/LAMA → combination
R	Review symptoms + exacerbations	Frequent exacerbations → escalate
E	Escalate treatment	ICS only if ≥2 exacerbations/year
A	Action plan	Written plan for worsening
T	Treat comorbidities	AF, heart failure, depression, osteoporosis
H	Healthy lifestyle + pulmonary rehab	MRC grade 3–5 → refer
E	Every review check technique	Always — no exceptions

Rescue Packs: ≥2 exacerbations/year → consider pre-supplied steroids + antibiotics with clear instructions.

Pulmonary Rehab: "Pulmonary rehabilitation can improve breathlessness, exercise tolerance, and reduce hospital admissions."

8 Smoking · Preventative Care · DVLA · Follow-Up

Ask–Advise–Act:

Ask: "Do you currently smoke or vape?"

Advise: "Stopping smoking is the single most effective thing you can do for your lungs."

Act: Offer nicotine replacement · Stop smoking referral · Medication support

Vaping Script: "Vaping is less harmful than smoking and may help some people quit, but the goal is to become nicotine-free."

Follow-Up	Interval
Stable	Annual review
Treatment changed	4–6 weeks
Poor control	Earlier review

Preventative Care: Annual flu vaccination · Pneumococcal vaccination · Smoking cessation support

DVLA: Consider if disabling symptoms / severe breathlessness / long-term oxygen therapy

Occupational trigger: "Do your symptoms improve when away from work?"